

When does this patient go home?

A quality improvement study compared two ways of answering it at Houston Methodist, and reported which one was closer.

Two answers, one date

CASE MANAGERS

AI ESTIMATIONS

The hospital’s own staff, and a commercial clinician-assisted tool integrated with the EHR, over inpatient encounters discharged in the window below.

AUGUST 1ST, 2023

FEBRUARY 28TH, 2024

At admission, level

At admission the case managers and the tool sat within a fraction of a day of each other on error.

CASE MANAGERS

4.20

AI ESTIMATIONS

4.27

MEAN ABSOLUTE ERROR IN DAYS, AT ADMISSION

The case managers did match the exact date more often, 23.6% against 15.3%.

On the last day, they were not

■ CASE MANAGERS 79.5%



■ AI ESTIMATIONS 37.9%



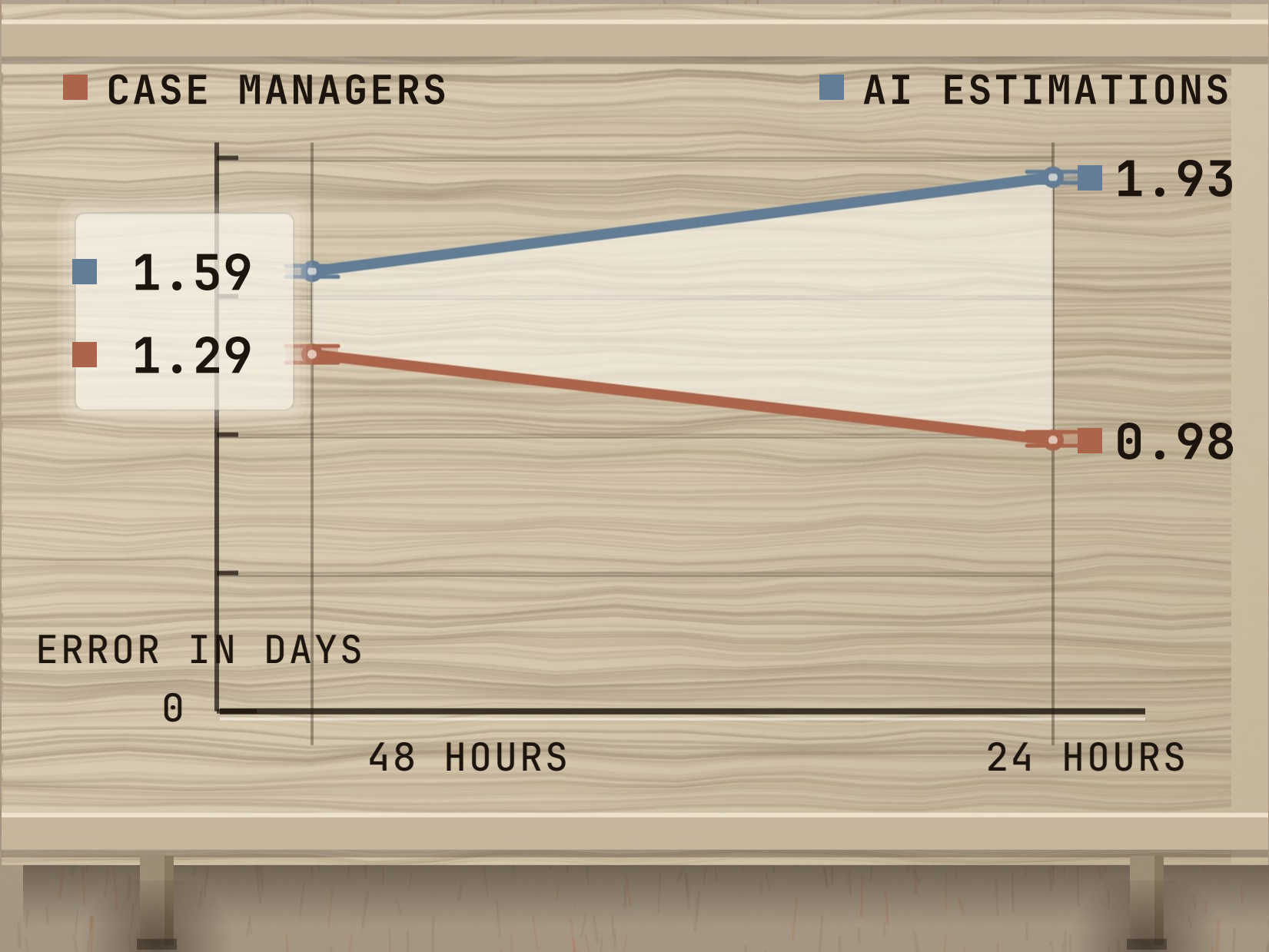
WITHIN ONE DAY OF THE TRUE DATE
24 HOURS BEFORE DISCHARGE

The authors say this is the point where an accurate date counts for most in bed management.

One improved. One got worse.

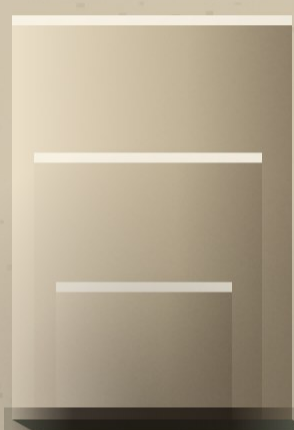
Mean absolute error in days. The two never meet.

HIGHER IS FURTHER FROM THE TRUE DATE



The strong admission correlation did not persist nearer discharge and should not be interpreted as evidence that the tool captured the same information clinicians use throughout the stay.

The study's authors, on their own result



**Agreement is not
understanding**

One estimate was being read

THE STUDY'S AUTHORS, AGAINST THEIR OWN RESULT

CASE MANAGERS

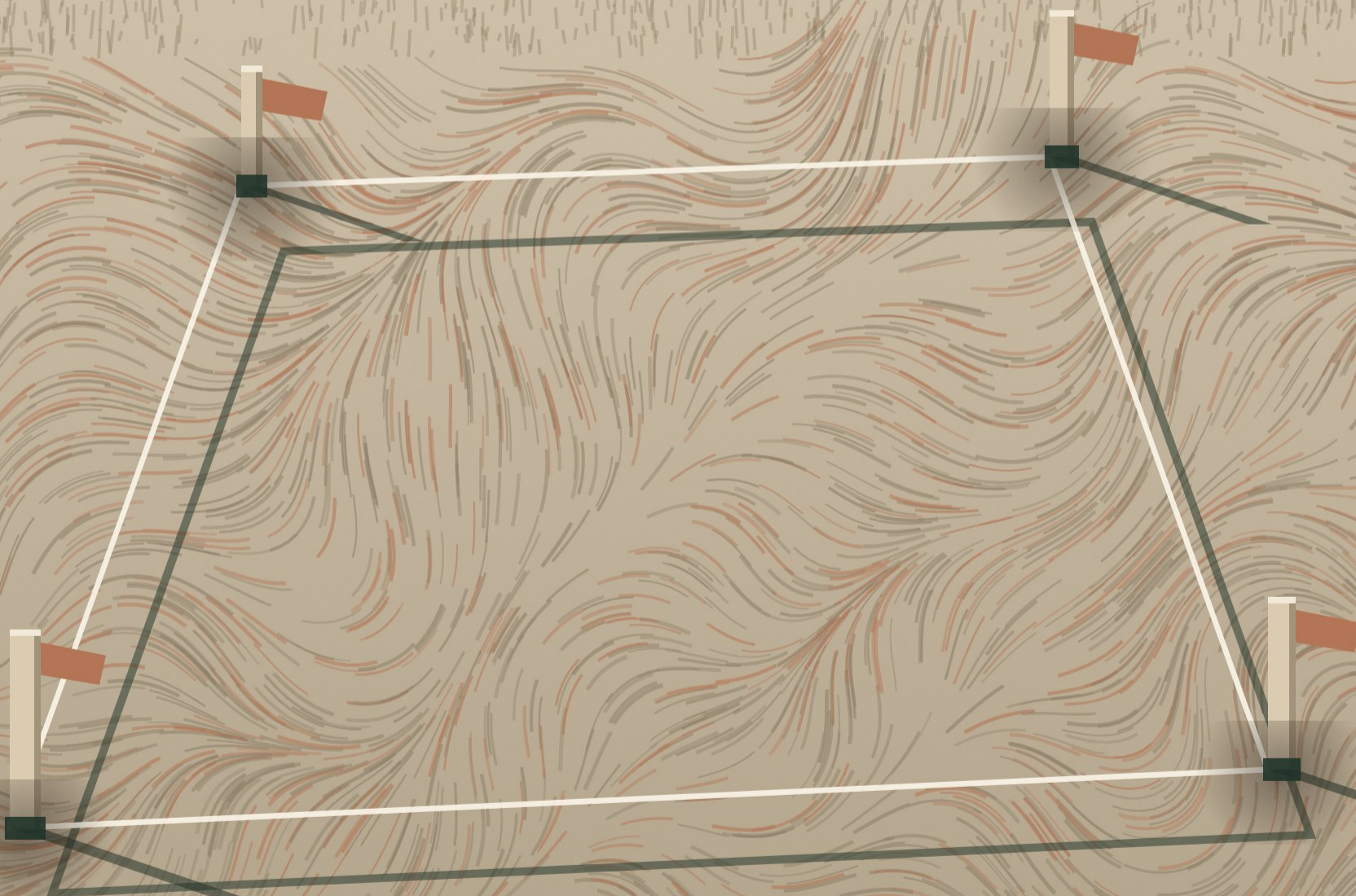
Estimates were part of usual care and visible to care teams. They may be partly self-fulfilling. The authors' own word for it is *advantaged nearer discharge*.

AI ESTIMATIONS

Accessible in the EHR but not enabled automatically.

What the comparison never saw

*No baseline patient demographic data were
available.*



THE STUDY FOLLOWED THE SQUIRE REPORTING GUIDELINE

What settles it

The authors say what has to happen before anyone decides the tool's extra accuracy in some subgroups justifies the complexity of putting it in.

Warrants prospective, outcome-based evaluation.

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